

DAY 24: STREAMLINING HOSPITAL DISCHARGE AND TAILORED PATIENT FEEDBACK

Cutting Discharge Time to 1 Hour using PDCA, Gemba Walk, FMEA and Patient Specific QFD

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Document Objective: Limit hospital discharge turnaround time from 4 to 6 hours down to strictly 1 hour. This operational framework combines PDCA driven Kaizen, Gemba Walk, Process and System FMEA, Kanban tracking, and Patient Specific Quality Function Deployment (QFD) to eliminate billing bottlenecks and capture patient centric feedback.

1. EXECUTIVE SUMMARY AND PROBLEM CONTEXT

Extended discharge delays of 4 to 6 hours clog hospital beds and frustrate families. A major operational flaw is treating all patient feedback as uniform because even patients with the same medical condition experience vastly different operational issues.

- **1 Hour Discharge Target:** Streamlining physician summaries, pharmacy clearance, and billing to a strict 60 minute limit.
- **Feedback Homogeneity Gap:** Standard surveys miss individual patient perspectives, leading to incomplete SOPs that fail to prevent specific errors.
- **Process Blindspots:** Lack of real-time status tracking between nursing counters, billing, and pharmacy desks.

2. INTEGRATED TQM TOOLKIT FOR DISCHARGE OPERATIONS

- **PDCA Cycle and Kaizen:** Core execution engine to enforce the 1 hour discharge protocol across all counters.
- **Gemba Walk and FMEA:** Observing billing counters directly to identify failure points in processes (PFMEA) and IT software (SFMEA).
- **Kanban Visual Status Board:** Tracking active discharge files via color coded bins to prevent queue bottlenecks.
- **Patient Specific VoP and QFD:** Administering tailored physical feedback forms designed around individual patient experiences to build comprehensive, error proof SOP manuals.

3. EXECUTION ROADMAP: PDCA DISCHARGE PROTOCOL

1. PLAN (1 Hour Target and Root Cause Mapping)

Target a strict 1 hour discharge window. Conduct Gemba Walks and execute System and Process FMEA at billing and summary desks.

2. DO (Kanban and Patient Specific Forms)

Deploy Kanban visual bins for file tracking. Issue tailored physical 1 page feedback forms customized to capture individual patient perspectives.

3. CHECK (Log Audits and Feedback Analysis)

Audit billing timestamps against the 1 hour KPI. Analyze unique patient feedback variations to detect subtle operational flaws.

4. ACT (Comprehensive SOP Manual Creation)

Translate diverse VoP inputs through QFD into a master operational SOP manual, standardizing staff procedures to eliminate recurring errors.

4. PROCESS IMPROVEMENT AND FAILURE ANALYSIS MATRIX

Discharge Bottleneck	Gemba and FMEA Findings	TQM Solution	Action Directive
Summary and Billing Delays: Idle time exceeding 1 hour limit.	Process FMEA: Uncoordinated file handoffs across departments.	Kanban Visual Board	Use color coded status bins to enforce a 60 minute strict turnaround time.
Billing System Errors: Software glitches during final reconciliation.	System FMEA: High RPN in insurance claim processing.	System FMEA and Kaizen	Automate insurance documentation and fix software bottlenecks.
Generic Feedback Form: Fails to capture unique patient issues.	VoP Gap: Uniform surveys miss specific operational friction points.	Patient Specific VoP and QFD	Design tailored physical feedback forms and use insights to build comprehensive SOPs.

Epilogue: Achieving a strict 1 hour discharge cycle transforms hospital operations. By combining Gemba Walks, FMEA, Kanban visual tracking, and patient specific VoP inputs into a master SOP manual, hospitals eliminate operational errors and deliver truly personalized patient care.

Disclaimer: This document has been created independently as part of the #100DaysOfTQM challenge, based on personal knowledge, quality engineering principles, and practical research.